

Mr ANAND KUMAR OJHA		Collected at: Faridiya CC Buxar Ward 13 P Road Jahaj Ghat Buxat Bihar 802101 Ph 9470231631	Collected : 25/04/2026 09:21 AM
Age	: 51 Yrs		Reported : 26/04/2026 05:27 PM
Gender	: Male		Report Status : Final
PID	: 1204C009260425001375	Processed at: Pathkind Labs Ghazipur, S.No 40 Badi Bagh Lanka 233001, Ph 7827949733	Ref. By : Self
VID	: 1204C009260425000406		
Barcode: 80021317232 , 80021317233 , 80021317234			

Test Name	Result	Biological Ref. Interval	Unit
HEALTHKIND ACTIVE			
Complete Blood Count (CBC)			
Sample : Whole Blood, EDTA			
Haemoglobin (Hb)	12.00 L	13.00 - 17.00	gm/dL
Total WBC Count / TLC	6.40	4.00 - 10.00	thou/ μ L
Neutrophils	64.00	40.00 - 80.00	%
Lymphocytes	28.00	20.00 - 40.00	%
Eosinophils	4.00	1.00 - 6.00	%
Monocytes	4.00	2.00 - 10.00	%
Basophils	0.00	0.00 - 2.00	%
Absolute Neutrophil Count (ANC)	4096.00	2000.00 - 7000.00	/ μ L
Absolute Lymphocyte Count	1792.00	1000.00 - 3000.00	/ μ L
Absolute Eosinophil Count (AEC)	256.00	20.00 - 500.00	/ μ L
Absolute Monocyte Count	256.00	200.00 - 1000.00	/ μ L
Absolute Basophil Count	0.00	0.00 - 100.00	/ μ L
RBC Count	4.08 L	4.50 - 5.50	million/ μ L
PCV / Hematocrit	35.60 L	40.00 - 50.00	%
MCV	87.00	83.00 - 101.00	fL
MCH	29.30	27.00 - 32.00	pg
MCHC	33.60	31.50 - 34.50	gm/dL
RDW (Red Cell Distribution Width)	14.40	11.80 - 15.60	%
Platelet Count	162.00	150.00 - 410.00	thou/ μ L
MPV (Mean Platelet Volume)	10.50	6.80 - 10.90	fL
Nucleated RBC Count	0.00	0.00 - 0.00	/100 WBC
Method: HB By Photometric ,TLC/Neutrophils /Lymphocytes/Eosinophils/Monocytes/Basophils By Fluorescence flowcytometry & microscopy ,ANC/ALC/AEC/AMC/ABC/PCV/MCV/MCH/MCHC/RDW/MPV By Calculated ,RBC/Platelet Count By Impedance ,NRBC By Microscopy			



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Test Name	Result	Biological Ref. Interval	Unit
Fasting Plasma Glucose Sample :Plasma Fluoride - Fasting Method : Hexokinase	107.30 H	Normal : 74 - 99 Impaired Fasting Glucose : 100 - 125 Diabetes : >125	mg/dL



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Test Name	Result	Biological Ref. Interval	Unit
Lipid Profile Sample : Serum			
Total Cholesterol Method : Spectrophotometry	136.00	Desirable Level : < 200 Borderline : 200 - 239 High Risk : >= 240	mg/dL
Triglycerides Method : Spectrophotometry	169.70 H	Desirable : < 150 Borderline High : 150 - 199 High : 200 - 499 Very High : >= 500	mg/dL
LDL Cholesterol (Calculated) Method : Calculated	71.36	Optimal : < 100	mg/dL
HDL Cholesterol Method : Spectrophotometry	30.70 L	40.00 - 60.00	mg/dL
VLDL Cholesterol Method : Calculated	33.94	Desirable : 10 - 35	mg/dL
Total Cholesterol / HDL Ratio Method : Calculated	4.43 H	Low Risk : 3.3 - 4.4 Average Risk : 4.5 - 7.0 Moderate Risk : 7.1 - 11.0 High Risk : > 11.0	Ratio
LDL / HDL Ratio Method : Calculated	2.32	Low Risk : 0.5 - 3.0 Moderate Risk : 3.1 - 6.0 High Risk : > 6.0	Ratio
Non HDL Cholesterol Method : Calculated	105.30	0.00 - 130.00	mg/dL



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Test Name	Result	Biological Ref. Interval	Unit
Liver Function Test (LFT) Sample : Serum			
Bilirubin Total Method : Spectrophotometry	0.56	0.00 - 1.20	mg/dL
Bilirubin Direct Method : Spectrophotometry	0.26 H	0.00 - 0.20	mg/dL
Serum Bilirubin (Indirect) Method : Calculated	0.30	0.00 - 0.90	mg/dL
SGOT / AST Method : Spectrophotometry	17.30	0.00 - 35.00	U/L
SGPT / ALT Method : Spectrophotometry	21.00	0.00 - 45.00	U/L
AST / ALT Ratio Method : Calculated	0.82		Ratio
Alkaline Phosphatase (ALP) Method : Spectrophotometry	44.00 L	53.00 - 128.00	U/L
Total Protein Method : Spectrophotometry	6.09 L	6.40 - 8.30	g/dL
Albumin Method : Spectrophotometry	4.07	3.50 - 5.20	g/dL
Globulin Method : Calculated	2.02	1.90 - 3.70	gm/dL
Albumin Globulin A/G Ratio Method : Calculated	2.02	1.00 - 2.10	Ratio



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Test Name	Result	Biological Ref. Interval	Unit
Kidney Function Test Sample : Serum			
Blood Urea Nitrogen Method : Spectrophotometry	11.64	8.00 - 26.00	mg/dL
Blood Urea Method : UREASE-GLDH	24.90	21.00 - 43.00	mg/dL
Creatinine Method : Spectrophotometry	0.76	0.70 - 1.30	mg/dL
BUN Creatinine Ratio Method : Calculated	15.31	10.00 - 20.00	Ratio
Uric Acid Method : Spectrophotometry	8.20 H	3.50 - 7.20	mg/dL
Sodium Method : ISE	140.00	136.00 - 145.00	mmol/L
Potassium Method : ISE	4.20	3.50 - 5.10	mmol/L
Chloride Method : ISE	100.00	97.00 - 107.00	mmol/L
Total Protein Method : Spectrophotometry	6.09 L	6.40 - 8.30	g/dL
Albumin Method : Spectrophotometry	4.07	3.50 - 5.20	g/dL
Globulin Method : Calculated	2.02	1.90 - 3.70	gm/dL
Albumin Globulin A/G Ratio Method : Calculated	2.02	1.00 - 2.10	Ratio
Calcium Method : Spectrophotometry	8.90	8.60 - 10.20	mg/dL



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Test Name	Result	Biological Ref. Interval	Unit
Estimated Glomerular Filtration Rate (eGFR)			
Sample : Serum			
Age Method : Manual	51.00	-	Years
GFR Male Method : Calculated	108.82	30-59: Moderate decrease in GFR 15-29: Severe decrease in GFR <15: Kidney failure >90: Normal GFR 60-89: Mild decrease in GFR	mL/min/1.73 m2

Authenticated by



Dr. Danish Ali
Danish Ali Rg-99907
(DMC)
MBBS MD
Regn # DMC 99907



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Processed at: Pathkind Labs Varanasi,
D63/8 1 Krishna Complex Mahmoorganj
221010, Ph 7827949764

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Test Name	Result	Biological Ref. Interval	Unit
HEALTHKIND ACTIVE			
 HbA1C (Glycosylated Hemoglobin) Sample : Whole Blood, EDTA Method : High Performance Liquid Chromatography (HPLC)			
HbA1c	5.60	Non Diabetic : 3.9 - 5.6 Pre Diabetic Range : 5.7 - 6.4 Diabetic Range : ≥ 6.5 Goal of Therapy : < 7.0 Action Suggested : > 8.0	%
Mean Plasma Glucose	114.02	0.00 - 116.00	mg/dL
 Gamma-Glutamyl Transferase (GGT) Sample : Serum Method : G- Glutamyl-Carboxy- Nitroanilide			
	17.30	10.00 - 71.00	U/L
Thyroid Profile Total Sample : Serum Method : ECLIA			
Total T3 (Triiodothyronine)	0.96	0.80 - 2.00	ng/mL
Total T4 (Thyroxine)	6.76	5.10 - 14.10	µg/dL
TSH 3rd Generation	2.150	0.27 - 4.20	µIU/mL
Phosphorus Serum Sample : Serum Method : Phospho Molybdate			
	3.61	2.50 - 4.50	mg/dL



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Authenticated by



Dr. Richa Katiyar
MBBS MD (Pathology)
Cluster Lab Head

Note: Tests marked with NABL symbol are accredited by NABL vide Certificate No. MC- 7993

Interpretation

Liver Function Test (LFT)

Indications for liver function assessment includes:

- Screen for liver infections, such as hepatitis
- Monitor the progression of a disease, such as viral or alcoholic hepatitis, and determine how well a treatment is working
- Measure the severity of a disease, particularly scarring of the liver (cirrhosis)
- Monitor possible side effects of medications

Kidney Function Test

Kidney function tests (KFT) are usually ordered when a patient has risk factors for kidney dysfunction such as high blood pressure (hypertension), diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening; screening patients at risk of developing kidney disease; management of patients with known kidney disease.

Lipid Profile

Lipid Profile consist of Triglycerides, Cholesterol and other lipoprotein fractions in serum. The levels reflect the status of Lipid metabolism in the body, collectively they aid in the diagnosis of various abnormal hyper lipidaemias. Analysis of Lipids has assumed greater importance due to increasing prevalence rates of Ischaemic Heart Diseases (IHD).

NCEP (ATP III) Guidelines.

Phosphorus Serum

Phosphorus deficiencies (hypophosphatemia) may be seen with malnutrition, malabsorption, acid-base imbalances, increased blood calcium, and disorders that affect kidney function. And phosphorus excesses (hyperphosphatemia) may be seen with increased intake of the mineral, low blood calcium, and kidney dysfunction.

Thyroid Profile Total

- Patient preparation is particularly important for hormone studies, results of which may be markedly affected by many factors such as stress, position, fasting state, time of the day, preceding diet & drug therapy.
- T3 is one of the thyroid hormones derived due to peripheral conversion of T4. The levels of T3 helps in the diagnosis of T3 Thyrotoxicosis and monitoring the course of hypothyroidism. However, T3 is not recommended for diagnosis of hyperthyroidism as decreased values have minimal clinical



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significance. Values below the lower limits can be caused by a number of conditions including non-thyroidal illness, acute and chronic stress and hypothyroidism. - Elevated level of T4 is seen in hyperthyroidism, pregnancy, euthyroid patients with increased serum TBG. Decreased levels are noted in hypothyroidism, hypoproteinemia, euthyroid sick syndrome, decrease in TBG. - TSH controls biosynthesis and release of thyroid hormones T3 & T4. TSH levels are increased in primary hypothyroidism, insufficient thyroid hormone replacement therapy, Hashimotos thyroiditis, use of amphetamines, dopamine antagonists, iodine containing agents, lithium, and iodide induced or deficiency goiter. Estimated Glomerular Filtration Rate (eGFR) eGFR is a test used to assess kidney function. It is not used to diagnose any disease. The eGFR test involves a blood test which measures creatinine. The eGFR is calculated from age, sex and blood creatinine level. EGFR calculation is provided only for adults more than 18 years of age. Complete Blood Count (CBC) CBC comprises of estimation of the cellular componenets of blood including RBCs, WBCs and Platelets. Mean corpuscular volume (MCV) is a measure of the size of the average RBC, MCH is a measure of the hemoglobin content of the average RBC and MCHC is the hemoglobin concentration per RBC. The red cell distribution width (RDW) is a measure of the degree of variation in RBC size (anisocytosis) and is helpful in distinguishing between some anemias. CBC examination is used as a screening tool to confirm a hematologic disorder, to establish or rule out a diagnosis, to detect an unsuspected hematologic disorder, or to monitor effects of radiation or chemotherapy. Abnormal results may be due to a primary disorder of the cell-producing organs or an underlying disease. Results should be interpreted in conjunction with the patient's clinical picture and appropriate additional testing performed. HbA1C (Glycosylated Hemoglobin) Hemoglobin A1c (HbA1c) level reflects the mean glucose concentration over the previous period (approximately 8-12 weeks) and provides a much better indication of long-term glycemic control than blood and urinary glucose determinations. American Diabetes Association (ADA) include the use of HbA1c to diagnose diabetes, using a cutpoint of 6.5%. The ADA recommends measurement of HbA1c 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to assess whether a patient's metabolic control has remained continuously within the target range. Falsely low HbA1c results may be seen in conditions that shorten erythrocyte life span. and may not reflect glycemic control in these cases accurately.			

** End of Report **

